

Bed Huddle and Capacity Escalation Guideline — Hospital Fort Bend

Tallowbrook Health | Nursing, Hospital Fort Bend | 2024-11-01

Guideline NS-FB-22, version 2.0, effective November 1, 2024. This guideline sets out the daily bed huddle and the three capacity levels used at Tallowbrook Hospital Fort Bend. It replaces the March 2021 version, which relied on ad hoc calls to the house supervisor.

Purpose

Hospital Fort Bend has 96 staffed beds on four units: ICU (12), 2 North Medical-Surgical (32), 3 North Medical-Surgical/Telemetry (30) and 3 South Surgical (22). Because the hospital is small, a few unexpected admissions can fill a unit quickly. The capacity levels give everyone the same signal and the same next steps.

Daily 10:00 huddle

The house supervisor leads a 15-minute huddle at 10:00 a.m. every day with unit charge nurses, case management, environmental services and the ED charge nurse. Each unit reports open beds, expected discharges and pending admissions from the EHR bed board. The house supervisor declares the capacity level for the day and re-declares it whenever conditions change.

Green level

Green applies when occupancy is below 80% and no admitted patient has waited in the ED more than two hours for a bed.

Routine placement by the house supervisor.

Case management confirms expected discharges by noon.

Elective surgical admissions proceed as scheduled.

Yellow level

Yellow applies when occupancy is 80% to 90%, or two or more admitted patients have waited in the ED more than two hours.

House supervisor notifies the chief nursing officer and the ED medical director.

Case management and hospitalists review all expected discharges before 11:00 a.m.

Environmental services prioritizes discharge cleans over routine work.

House supervisor arranges additional nursing staff through the unit call-off list or overtime.

Red level

Red applies when occupancy is above 90%, the ICU has no open bed, or four or more admitted patients have waited in the ED more than four hours.

Chief nursing officer or administrator on call joins a capacity call within 30 minutes.

Elective inpatient surgical cases for the next day are reviewed with Perioperative Services.

Transfers of suitable patients to Medical Center West are considered with the West capacity manager.

Diversion is requested only by the administrator on call with the ED medical director.

Transfers to Medical Center West

Patients needing services not available at Hospital Fort Bend, including interventional cardiology for STEMI and obstetric care, are transferred to Medical Center West under existing protocols regardless of capacity level. Capacity-related transfers require acceptance by a West physician and the West house supervisor.

Review

The nursing leadership team reviews the thresholds annually or after any red-level day that lasts more than 12 hours. Questions go to the Hospital Fort Bend chief nursing officer.